



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°127/MVEBDB/09/18/2026

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HIGHLIGHTS (last 24 hours)

Over the past 24 hours, 73 new confirmed cases of Ebola Virus Disease (EVD), including 25 community deaths, have been reported in four provinces: Ituri (46 cases), North Kivu (18 cases), Haut-Uélé (6 cases), and Tshopo (3 cases). In addition, 12 deaths in Ebola Treatment Centers (ETCs) have been recorded.

A new health zone has been affected in the last 24 hours, namely Dungu in the Haut-Uélé province.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
7,614	3,676	48.3%	909	1864	87.8%



Affected Provinces and Affected Health Zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé and South Ubangi



63 out of 167 health zones affected (37.7%)

Summary of key points from the last 24 hours

A total of **73 new confirmed cases** of Ebola Virus Disease (EVD) were reported as of September 18, 2026, including **25 community deaths**. Of these confirmed cases, **46** were from Ituri province, **18** from North Kivu, **6** from Haut-Uélé, and **3** from Tshopo.

A new health zone, Dungu, in Haut-Uélé province, was affected during the day. As this zone borders South Sudan, increased cross-border surveillance is necessary.

During the day of September 18, 2026, **12 intra-CTE deaths** (10 in Ituri and 2 in North Kivu) were unfortunately recorded, bringing the cumulative number of deaths for the day to **37**.

However, **41 patients** were declared cured (**30** in Ituri, **6** in Haut-Uélé, **3** in Nord-Kivu and **2** in Tshopo) after obtaining two negative results for Ebola virus disease.

Since the start of the epidemic in the Democratic Republic of Congo (DRC), a cumulative total of **7,614 confirmed cases**, including **3,676 deaths**, has been recorded, representing a case fatality rate of **48.3 %**. The epicenter remains the Ituri province, which accounts for **77.2%** of the confirmed cases reported in the seven affected provinces and also accounts for **63.0 %** of new confirmed cases.

A new health zone was affected during the day on September 18, 2026. Thus, **63 out of 167 health zones (37.7%)** are affected in **7 provinces**. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (7/13), Tshopo (7/23), South Kivu (1/34), Bas-Uélé (3/11) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 18, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	46	5875	2,688	45.8%	28/36 (77.8%)
North Kivu	18	1388	843	60.7%	16/34 (47.1%)
Haut-Uélé	6	302	126	41.7%	7/13 (53.8%)
Tshopo	3	38	13	34.2%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	7	4	57.1%	3/11 (27.3%)
South Ubangi	0	1	1	100.0%	1/16 (6.3%)
Total	73	7,614	3,676	48.3%	63/167 (37.7%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 18, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5875	2688	45.8%	46	17	10	27
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	187	39	20.9%	1	0		0
Boga	2	2	100.0%				0
Bunia	1646	513	31.2%	11	1		1
Damascus	40	9	22.5%				0
Drodro	15	7	46.7%				0
Fataki	80	30	37.5%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	34	12	35.3%				0
Komanda	146	89	61.0%	5	1		1
Lita	272	135	49.6%	3	2		2
Logo	13	5	38.5%				0
Lolwa	31	8	25.8%				0
Mahagi	4	1	25.0%				0
Mambasa	21	9	42.9%				0
Mandima	59	33	55.9%				0
Mangala	331	146	44.1%	5	0		0
Mongbwalu	679	315	46.4%	3	3		3
Nia-Nia	262	140	53.4%	9	8		8
Nizi	749	274	36.6%	1	0		0
Nyankunde	126	36	28.6%	1	0		0
Rimba	12	4	33.3%				0

Rwampara	1043	352	33.7%	7	2		2
Tchomia	75	25	33.3%				0
To ventilate	N / A	487	N / A			10	10
North Kivu	1388	843	60.7%	18	4	2	6
Blessed	271	186	68.6%	8	0		0
Good	18	7	38.9%				0
Butembo	249	161	64.7%	3	0	1	1
Goma	1	0	0.0%				0
Kalunguta	41	14	34.1%	1	0	1	1
Katwa	540	342	63.3%	2	2		2
Kayna	1	0	0.0%				0
Kyondo	32	20	62.5%				0
Lubero	15	7	46.7%				0
Mabalako	24	11	45.8%	1	1		1
Manguredjipa	10	3	30.0%				0
Masereka	24	9	37.5%				0
Musienene	120	65	54.2%	1	1		1
Mutwanga	8	2	25.0%				0
Oicha	13	10	76.9%	1	0		0
Vuhovi	21	6	28.6%	1	0		0
Haut-Uélé	302	126	41.7%	6	3	0	3
Boma Mangbetu	36	15	41.7%	1	0		0
Dungu	1	1	100.0%	1	1		1
Gombari	3	1	33.3%				0
Isiro	98	42	42.9%				0
Pawa	67	32	47.8%	4	2		2
Rungu	2	1	50.0%				0
Wamba	95	34	35.8%				0
Tshopo	38	13	34.2%	3	1	0	1
Bafwasende	7	1	14.3%				0
Kabondo	5	1	20.0%	1	0		0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	19	8	42.1%	2	1		1
Mangobo	4	2	50.0%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas-Uélé	7	4	57.1%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	3	1	33.3%				0
Ganga	3	2	66.7%				0
South Ubangi	1	1	100.0%	0	0	0	0
Bulu	1	1	100.0%				0
Total	7614	3676	48.3%	73	25	12	37

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

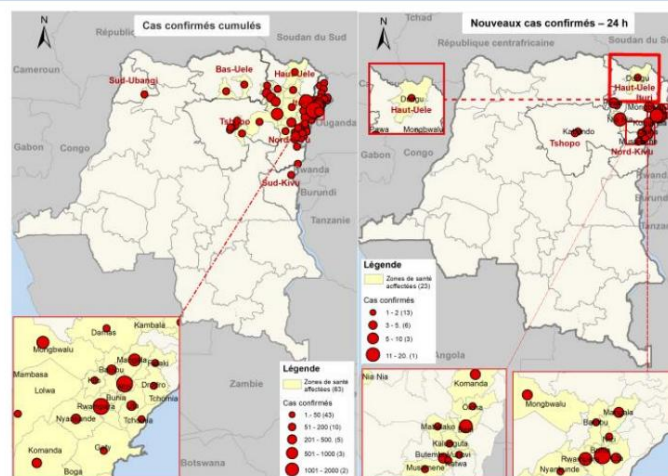
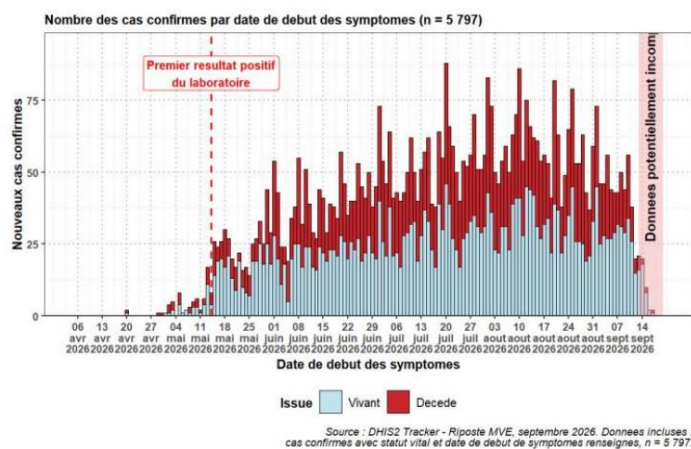
NA: Not Applicable.

Table 3. Status of alerts notified by province for September 18, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CTE/CT (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	45	4	49	19	3	24	1	22	9
Bas-Uélé	74	2	76	7	2	66	0	9	0
Ituri	1108	92	1200	143	89	667	2	232	118
North Kivu	373	24	397	62	24	255	0	86	52
South Ubangi	45	5	50	3	0	0	0	3	1
Tshopo	75	5	80	5	3	72	0	8	3
Total General	1,720	132	1,852	239	121	1084	3	360	183

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

Based on the reported data, **1,852 alerts** were received, of which **1,447 (78.1%)** were verified.

Among these, **360 alerts** were validated as suspected cases.

All of these suspected cases (100%) were investigated, and **183** were transferred to CT/CTE (Table 3).

Over the past 24 hours, **26,803 of the 30,541 contacts** currently being monitored were seen, representing a follow-up rate of **87.8%**, stable compared to the previous day (87.2%). This rate was 100.0% (42/42) in South Ubangi, 94.0% (13,964/14,861) in Ituri, 91.5% (193/211) in Bas-Uélé, 82.2% (11,345/13,801) in North Kivu, 78.0% (981/1,257) in Haut-Uélé and 75.3% (278/369) in Tshopo.

1.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.1. Main actions

Six (6) alerts were notified to the various PoE/PoC of the five monitored provinces during the last 24 hours. Four (4) alerts were reported in Ituri and two (2) in North Kivu; all validated alerts were investigated within 24 hours (100%; 5/5) and three were transferred to the CT/CTE (60.0%; 3/5).

Table 4. Status of key PoE/PoC indicators by province, as of September 18, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	95.0% (57/60) 100%	(20/20) 55.6% (5/9)	100% (18/18) 71.4%	(5/7) 92.1% (105/114)		
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%	100%
Number of people who have passed PoE/PoC	146,383	93,658	8,152	19,745	13,207	281,145
% of travelers screened	96.0%	97.7%	98.5%	99.3%	99.0%	97.0%
% of travelers who washed their hands	96.0%	97.7%	98.7%	99.2%	99.0%	97.0%
% of travelers made aware	99.0%	97.7%	100.0%	100.0%	100.0%	98.7%
Number of alerts notified	4	2	0	0	0	6
Number of validated live alerts (suspected cases)	3	2	0	0	0	5
Number of bodies intercepted today % of suspected	0	0	0	0	0	0
cases transferred to CTE/CT	33.3% (1/3)	100% (2/2)	N / A	N / A	N / A	60.0% (3/5)
% of lifeless bodies swabbed/secured	N / A	N / A	N / A	N / A	N / A	N / A
% of suspected cases investigated within 24 hours	100% (3/3)	100% (2/2)	N / A	N / A	N / A	100% (5/5)

1.2.2. Challenges

Suboptimal operationalization of PoE/PoC coupled with low reporting completeness in South Kivu (55.6%, 5/9); persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri), low alert coding in DHIS2 Tracker, and insufficient mobility in Rwampara and Bunia. In North Kivu, lack of support from security personnel at the Butembo axis PoCs and threats against service providers. In South Kivu, absence of isolation measures at the Kavimvira, Kalundu Port SNCC, and Baraka PoEs.

1.3. Laboratory

1.3.1. Main actions

- **Ituri: 46 new positive results** (29 living and 17 deaths) out of 200 new samples received and analyzed (**positivity: 23.0%**).
- **North Kivu: 18 samples tested positive** (14 live and 4 dead) out of 116 samples analyzed (**positivity of 15.5%**).
- **Haut-Uélé: 6 new positive results** (3 living and 3 deceased) out of 34 samples received and analyzed at the Isiro laboratory (**positivity: 17.6%**).
- **Tshopo: 3 new positive results (2 alive and 1 dead) out of 6 samples received and tested (positivity: 50%).**
- **Bas-Uélé: 4 samples received and analyzed (4 live) all tested negative.**

- **South Ubangi:** samples taken around the two reported community deaths have been sent to the INRB and the results are awaited.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri,** 27 rings were opened, representing an opening rate of 61.3%, and 69 households were decontaminated out of the 72 identified (95.8%); 13 health facilities were decontaminated, but no hygiene kits were distributed to households or schools in the last 24 hours. Regarding blood and sanitation surveys (BSS), 104 alerts were recorded, 80 BSS were carried out (76.9%), and 69 bodies were swabbed out of 80 (86.3%).
- **In North Kivu,** 31 of the 40 planned infection control rings were opened (77.5%), 5 of the 15 identified households were decontaminated (33.3%), and 24 of the 29 health facilities were decontaminated (82.8%). No infection prevention and control (IPC) kits were provided to households of cases, and 12 scorecard assessments were conducted, with an average score of 17.6%. Nineteen health facilities were assessed, 279 received support, 879 service providers were briefed, 180 handwashing stations were monitored, 3 triage units were installed, and 10,600 liters of water were provided to public places. Regarding blood and body disposal (BBD) testing, 53 BBDs were conducted, and 42 of the 44 bodies were swabbed (95.5%).
- **In Tshopo,** second day of training for 95 PPLs from the health zones of Kabondo and Makiso-Kisangani; assessment of the risk of exposure of PPLs from the DHK (10 at high risk, 14 at low risk and 3 at undetermined risk); decontamination of the HGRs of Makiso-Kisangani and Mangobo as well as six premises of two households; monitoring and support of 13 ESSs in the 6 health zones with 139 briefed providers.

As for the EDS, 1 lifeless body was swabbed at the Rekapi morgue.

- **In Haut-Uélé,** 6 of the 6 planned testing sites were opened (100%), and 7 households were decontaminated; 17 infection prevention and control (IPC) alerts were recorded in the last 24 hours. Regarding the health and safety surveys (HSS), 4 alerts were recorded, 4 HSS were completed (100%), and 4 bodies were swabbed (100%).
- **In Bas-Uélé,** training continues for the 6 service providers from the 3 PoCs in the Buta health zone with the support of FHI 360 and the guidance of the ESS in the health zones of Ango, Monga and Ganga.
- **In South Ubangi,** 1 out of 2 identified households (50.0%) was decontaminated in the Gwaka health area, without any open rings or decontaminated health facilities; monitoring and support were provided to healthcare providers at the Kuma and Akula health centers, and on-site briefings were given to 2 laboratory technicians and 10 hygiene workers on the donning and doffing of PPE, sample collection at suspected deaths, and blood and body screening (BBS). Regarding BBS, 2 alerts were recorded, 1 BBS was performed, and 1 body was swabbed.

1.4.2. Challenges

- **In North Kivu,** low coverage of household decontamination (33.3%; 5/15) and ring opening still incomplete (77.5%; 31/40); decontamination teams not trained or activated in the health zones of Biena, Masereka, Kyondo, Mabalako and Vuhovi; non-alignment of 86.6% of PCI/WASH pillar members on the motivation list; lack of mobility for the deployment of provincial PCI teams, lack of traceability of some eligible households and need for training of PCI data managers.
- **In Tshopo,** there is a shortage of IPC supplies in households, schools, markets, and churches; a low proportion of social and solidarity economy (SSE) workers have an IPC score above 80% (2.2%; 2/93), and only partial coverage of SSE workers with IPC kits (47.3%; 44/93); the incinerator at the Makiso-Kisangani General Referral Hospital remains out of service due to a lack of technicians. In Haut-Uélé, there are logistical constraints,

Lack of computers and need for training of service providers and hygiene workers; refusal of swab by some families and threats against response teams in Isiro.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, 102 new admissions were recorded during the day, compared to 98 discharges, including 30 recoveries. Thus, 434 patients are hospitalized out of 1015 beds, representing an occupancy rate of 42.8%; 390/421 (92.6%) patients received a hot meal; 7/7 children are receiving LNPE/SLM at the CTE, 4/4 at the nursery, and 25/25 in the community; 40 patients are in critical condition, and 22 patients were evacuated during the 17/33 reported ambulance movements.
- **In North Kivu**, 70 new admissions were recorded, compared to 69 discharges, including 3 recoveries; 392 patients are hospitalized, including 224 in standardized facilities with a capacity of 228 beds, representing an overall occupancy rate of 98.2%, close to saturation. All 4 caregivers and 77 frontline providers received hot meals, all 11 separated infants are receiving LNPE/SLM, 3 children are attending daycare, and all 20 patient households received food assistance; 18 ambulance trips were made, all successful (100%); 11 patients are in critical condition.
- **In Haut-Uélé**, 14 new admissions were recorded in the facilities compared to 16 discharges (6 recovered). Thus, 63 patients are hospitalized for 138 beds, representing an occupancy rate of 45.7%; 30 confirmed, 30 suspected, 79 accompanying persons and 36 PPLs received hot meals; 9 ambulance trips were made for 9 transported patients.
- **In Bas-Uélé**, 1 new admission reported; 10 patients remain in isolation including 1 confirmed case taken care of at the HGR of Ganga; 9 high risk contacts remain under prophylaxis and 10 have left unscathed from the follow-up.
- **In Tshopo**, 2 new admissions were recorded, compared to 4 discharges, including 2 recoveries and 2 non-cases. Ten (10) patients are in isolation out of 31 beds, representing an occupancy rate of 32.3%; 8 confirmed cases and 1 suspected case received a hot meal and 42/42 (100%) frontline providers were served.

1.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 42.9% of hospitalized patients are treated outside of standardized facilities; no beds are available in Butembo and Katwa. **In Tshopo**, the lack of safe blood and limited technical resources persist. **In Bas-Uélé**, the absence of ETCs and ambulances hinders the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 3,369 people have been vaccinated to date out of 11,703 targeted TPL/PPL (28.8%), including 1,830 in Makiso-Kisangani, 466 in Kabondo, 422 in Mangobo, 316 in Tshopo, 173 in Bafwasende and 162 in Lubunga; the stock available at the Hub depot amounts to 500 frozen doses and 631 thawed doses expiring on September 24, 2026.
- **In Ituri**, preparations continue for the official launch of the vaccination campaign for PPL, scheduled for September 19, 2026 at the Grand Laboratory in Bunia with the support of MSF; the launch site is ready.
- **In Bas-Uélé**, compassionate vaccination continues in Ganga, and the transport of cold chain equipment to the EPI (Expanded Program on Immunization) unit in Buta is still underway. In North Kivu, cold chain equipment has been received in preparation for the vaccination campaign: 2 refrigerators, 2 freezers, 1 stabilizer, and 10 vaccine carriers. **In Ituri**, preparations for vaccinating people living with HIV/AIDS continue.

1.6.2. Challenges

Low adherence of PPL and TPL to vaccination in Tshopo, where coverage of the target remains at 28.8% and remains particularly low in Lubunga (10%), Mangobo (18%) and Bafwasende (19%); risk of expiry of the 631 doses thawed by September 24, 2026.

In Bas-Uélé, there is no microplan for the vaccination of PPL in the six priority health zones (Aketi, Ango, Bili, Buta, Ganga and Viadana).

1.7. Continuity of Care

- **In Tshopo**, 146 cases received the experimental treatment, including 19 confirmed and 127 under investigation Post-exposure prophylaxis (92 completed, 11 interrupted and 24 ongoing); the 7th and 8th PPLs were confirmed positive and at least 88% of the PPLs in the visited ESS are vaccinated (21/24); a review of maternal deaths from eclampsia was organized by the central office of the health zone.
- **In Bas-Uélé**, continued awareness-raising of contacts for adherence to compassionate prophylaxis in Viadana and the search for contacts never seen, on day 35 in Buta and on day 23 in Ganga, Poko and Viadana.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, 17,802 home visits reached 81,306 people and 164 educational talks reached 748 people; 12 mass awareness campaigns reached 2,496 people, 9 community debates reached 208 people and 9 advocacy campaigns reached 14 people; 217 community alerts were notified (including 51 deaths), 48 of the 68 feedbacks were resolved within 48 hours and 26 of the 36 community incidents as well; 60 awareness spots were broadcast on 15 radio stations; the completeness of reporting remains low (39%; 11/28 ZS).
- **In North Kivu**, 10,508 home visits reached 29,430 people and 139 educational talks reached 3,363 people; 9 mass communication campaigns reached 1,188 people and 3 briefings reached 112 people, while 397 students and 49 teachers were sensitized; 300 community alerts were notified out of 450 expected (67%), 11 feedback issues were resolved out of 16 (69%), 1 incident was resolved out of 6 (16.7%), and 1,205 people were engaged out of 4,750 (25%); a community dialogue brought together 104 opinion leaders in Manguredjipa regarding the reintegration of the first person to recover from HIV in the area, and the discharge of 4 recoveries from the Ebola Treatment Center (ETC) in Beni was publicized; coverage remains at 10 out of 14 expected health zones (71.4%).
- **In Haut-Uélé**, 13 community engagement activities reached 275 people (101 men and 174 women) and actions carried out around the last confirmed case in Viadana reached 7 people; 21 alerts of suspected cases out of 160 expected (13.1%) and 4 alerts of community deaths (2.5%) were notified by community actors, 11 feedbacks were resolved within 48 hours and 1 incident out of 98 (1.0%).
- **In Tshopo**, 3 mass awareness activities reached 1,978 people (1,081 men and 897 women), 129 posters and 5 banners were distributed and 4 radio broadcasts were aired; 4 community alerts were notified and all resolved, as well as 2 feedbacks within 48 hours.
- **In South Ubangi**, 358 home visits reached 1,790 people (806 men and 984 women) in the health areas of Gwaka, Kuma and Akula; no community alerts were notified and the CREC supported the surveillance, laboratory, IPC and EDS pillars around the community death in Akula as well as the psychosocial support of the family of the confirmed case in isolation.

1.8.2. Challenges

Persistent community resistance: in Tshopo, the confirmed case of escape from the CTE (20th cases) remain at home, refusals to list contacts and investigate suspects are reported to DHK around the 29th case and 5 resistances have been notified of which only 2 were broken.

In Ituri, low data reporting in some health zones (completeness: 8/28; 29%), absence of

Numerical data on interventions around cases and demotivation of stakeholders due to unpaid bills. In North Kivu, refusal to register and decontaminate in Butembo and Biena, threats against an IPC worker, and denial of the disease. In Bas-Uélé, low reporting of community alerts, demotivation of CREC workers, and impassable roads.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 90 of the 141 confirmed cases under follow-up (63.8%) benefited from SMSPS interventions, as well as 28 of the 97 new suspects (28.9%) and 124 of the 141 suspects under follow-up (87.9%); 88 results announced, including 32 positive cases; 288 caregivers and 50 PPLs supported; 9/9 children followed in daycare; 17/21 recovered patients followed in the community; 11 of the 14 households of separated and orphaned children supported (34 children followed) and 9 of the 30 recovered patients reintegrated; deployment of 24 trained APS/TPS in the health zones of Nizi, Bambu, Lita and Nyankunde.
- **In North Kivu**, 20 people among the new confirmed cases and 215 confirmed cases under follow-up benefited from SMSPS interventions, as well as 217 new suspected cases and 276 suspected cases under follow-up; 113 results announced, including 27 positive cases; 78 households supported and 77 PPLs supported; 10/10 recovered, reintegrated and followed up in the community; 41/41 children followed up in the community, 3/3 in daycare and 20/20 households of separated and orphaned children supported; 125 men, 178 women and 69 children benefited from psychoeducation sessions with the support of UNICEF and all 8 notified health zones are covered (100%).
- **In Tshopo**, 4 of the last 5 confirmed cases (80%) and 2/2 confirmed cases under follow-up have benefited from SMSPS interventions, as well as 1/1 new suspect and 2/2 suspects under follow-up; 6 results announced, including 5 positive ones, 8 households supported, 1 PPL supported, 2/2 recovered and followed up in the community and 2 of the 3 resistances broken in the Kabondo health zone; 3 of the 4 notified health zones are covered (75.0%).
- **In Haut-Uélé**, 2 new confirmed cases and 16 confirmed cases under monitoring benefited from SMSPS interventions, as well as 4 new suspected cases and 10 suspected cases under monitoring; 12 results announced, including 2 positive results; 37 caregivers and 39 people with disabilities received support, and 15 household members received support; 50 recovered patients are still awaiting their unloading kits.

1.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions (10 of the 28 health zones covered). **In North Kivu**, Low coverage around the latest confirmed cases in nine health zones, lack of childcare facilities in 10 affected areas, and insufficient human resources. **In Tshopo**, there is no SMSPS (Social and Medical Prevention Service) provider in Wanierukula and Bafwasende. **In Haut-Uélé**, there are difficulties with the mobility of community health workers in Wamba and Boma Mangbetu, a lack of school and nutritional kits for separated children, and poor data reporting in Boma Mangbetu and Pawa. In Bas-Uélé, there is no psychologist in the Ganga health zone.

1.10. Logistics

1.10.1. Main actions

- Ensure the mobility of response teams in all affected provinces and continue construction work on the CTE at the Musienene General Referral Hospital in North Kivu.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu**, the pillar provided support and security coverage for IPC teams for the decontamination of households in the health districts of Mangote, Benegule, Kanzabi and Tuungane as well as the CH Sifa, CM La Brèche and CS La Charité facilities; the EDS teams were accompanied for the burial of confirmed cases at the family cemeteries of Mukulya and Bundji and for the swab of lifeless bodies at CH Maombi and Rugenda, with 100% coverage of the interventions requested.

1.10.4. Challenges

In North Kivu, no data has been transmitted by the security pillar in the last 24 hours.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions

- In Ituri, 49 staff members, including 17 women and 32 men, were briefed on the PSEAH in the health zones of Bunia, Nyankunde, Nizi, and Rwampara and all signed the code of conduct; 255 people, including 184 women, were sensitized in the community, and 3 partners (WHO, IRC, and Mercy Corps) reported on their PSEAH activities
- Haut-Uélé: 4 of the 7 health zones have carried out PSEA activities: 40 people sensitized in Isiro, 26 PRESICODESA in Gombari, 136 heads of schools in Wamba and 6 people in Pawa, 40 codes of good conduct having been signed; 6/7 health zones know the reporting mechanism (85.7%).
- In North Kivu, a briefing was organized for 52 participants, including 23 men and 29 women. In the Katwa Health Zone, Vungi Health Area, this was followed by community consultations in 4 focus groups, one for men and youth, one for women, and one for people with disabilities.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- A new health zone (Dungu) has been affected in Haut-Uélé province. The provinces of Ituri, North Kivu, and Haut-Uélé remain in a state of transmission sustained, a rebound was observed in the province of Tshopo while in Bas-Uélé, no confirmed cases were reported in the last 48 hours.
- In the last 24 hours, **73 new confirmed cases**, including **25 community deaths**. **On the other hand 12 intra-CTE deaths** were reported. Thus, the national cumulative total is **7,614 confirmed cases including 3,676 deaths (case fatality rate: 48.3%)**.
- Ituri remains the epicenter; it accounts for **77.2%** of confirmed cases and **63.0%** of new cases reported in the last 24 hours.
- The performance of epidemiological investigations remains satisfactory, with 100% of suspected cases investigated and a stable contact tracing rate of **87.8%**.
- Strengthening interventions (surveillance, care, IPC, logistics and community engagement) remain essential to quickly interrupt transmission chains: Rapidly increase reception capacities in ETCs/CTs in North Kivu and strengthen cross-border surveillance with South Sudan.

Priorities

essential to quickly interrupt transmission chains: Rapidly increase reception capacities in ETCs/CTs in North Kivu and strengthen cross-border surveillance with South Sudan.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

- Strengthen preparedness and response to Ebola Virus Disease-B in the provinces of the River Corridor and surrounding areas.



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